SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

AC → weekly paclitaxel + weekly trastuzumab

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

HER2-positive neoadjuvant or adjuvant high-risk breast cancer · NCCP 00432 · version 5 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment is part of a plan to reduce the chance of breast cancer returning.

Schedule
Four AC cycles 21 days apart, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total, without overlapping it with AC.

1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Doxorubicin
ANTHRACYCLINE CHEMOTHERAPY

- ☐ Low counts, infection and mouth ulcers
- ☐ Nausea, hair loss and fatigue
- ☐ Heart damage, sometimes delayed
- ☐ Severe tissue injury if the infusion leaks
- ☐ Rare later blood cancers

Cyclophosphamide
ALKYLATING CHEMOTHERAPY

- ☐ Low counts and infection
- ☐ Nausea, vomiting and hair loss
- ☐ Bladder irritation / bleeding
- ☐ Fertility effects and menopause
- ☐ Rare later blood cancers

Paclitaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Numbness / tingling, sometimes lasting
- ☐ Hair loss, muscle or joint pain
- ☐ Infusion reaction
- ☐ Mouth, bowel, skin and nail effects

Trastuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction / heart failure
- ☐ Infusion or allergic reaction
- ☐ Diarrhoea and fatigue
- ☐ Rare serious lung effects

Discuss treatment aim, alternatives, uncertainty, fertility, contraception, access, phase changes, serious and lasting risks, supportive medicines and the urgent contact plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00432 v5.

SACTCheck v0.84.0
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